INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Adjuvant ribociclib + endocrine therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00892
Version 2

> Oral treatment with a planned rest week

Treatment stages

01 **Ribociclib + endocrine therapy**
Up to 3 years

02 **Endocrine therapy continues**
Duration individually agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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04	Your record and contact card	12
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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Adjuvant ribociclib + endocrine therapy · National Cancer Control Programme (NCCP) 00892

How this medicine supports your treatment

For eligible higher-risk HR-positive/HER2-negative early breast cancer, ribociclib is added to endocrine therapy to reduce recurrence risk.

CDK4/6 INHIBITOR

Ribociclib

Slows the cell-division checkpoint

CDK4/6 checkpoint → Checkpoint inhibited → Cell cycling slows

Ribociclib blocks CDK4 and CDK6, proteins that help cells move through the cycle of division. Endocrine therapy reduces the hormone signal that can drive HR-positive breast cancer.

Keep blood and electrocardiogram (ECG) appointments. Fever, jaundice, fainting, palpitations, severe rash or new breathlessness need prompt advice.

ENDOCRINE PARTNER

Aromatase inhibitor ± ovarian suppression

Reduces oestrogen signalling

Oestrogen pathway → aromatase inhibitor (AI) or receptor blockade → Growth signal reduced

The aromatase inhibitor reduces oestrogen production. Before menopause, ovarian suppression is also required; men receiving this AI combination need the appropriate LHRH-agonist plan under product information. The endocrine treatment is taken on a separate schedule and normally continues after ribociclib.

Discuss joint symptoms, hot flushes, sexual symptoms and bone health. Keep ovarian-suppression appointments if prescribed.

Your treatment aim

Who this treatment is for

HR-positive/HER2-negative early breast cancer at high recurrence risk: stage IIA with qualifying nodal/high-risk factors, or stage IIB–III, as specified by NCCP. Current NCCP00892v2 includes selected node-negative eligibility and a conflicting node-negative exclusion; clinical reconciliation remains required.

NCCP 00892 v2 · official regimen

Schedule and duration

The usual adjuvant starting dose is ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to three years. The aromatase inhibitor continues every day on its own plan; ovarian suppression is added when indicated. At completion of the planned three-year ribociclib course, continue the separately agreed endocrine and follow-up plan.

NCCP 00892 v2 · official regimen

What the studies tell us

Ribociclib added to an aromatase inhibitor reduced the chance of invasive cancer returning in the study. Recent longer follow-up also reported a favorable survival finding, but that analysis was descriptive and needs that qualification. Your treatment team can explain how the study matches your recurrence risk.

How to interpret this

Study results describe groups with defined treatment and eligibility. Ask about your own likely benefits, important harms, alternatives and remaining uncertainty.

NATALEE primary report, Slamon et al 2024 · NATALEE prespecified five-year report, Crown et al 2025 · NATALEE six-year descriptive analysis, Slamon et al, Annals of Oncology journal pre-proof, 23 October 2026

NATALEE: primary trial and prespecified six-year descriptive follow-up

Studies describe groups. Ask how closely the treatment and study population match your situation.

Your team can discuss the strength and limitations of the evidence alongside treatment risks and alternatives.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Adjuvant ribociclib + endocrine therapy · NCCP 00892

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00892 v2 · official regimen · EMA Kisqali current product information; checked 2026-10-25

THE WHOLE TREATMENT JOURNEY

Adjuvant ribociclib + endocrine therapy: treatment and checks

Treatment stages

01 **Ribociclib + endocrine therapy**
Up to 3 years

02 **Endocrine therapy continues**
Duration individually agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Adjuvant · HR-positive / HER2-negative early breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before first dose	Cycles 1–2	Cycles 3–6	Later treatment	Completion / change
Ribociclib and endocrine partner NCCP 00892 · SMPC	Confirm adjuvant 400 mg / AI plan	Days 1–21 on; Days 22–28 off	Repeat if cleared; endocrine partner continues	Up to three years	Review endocrine continuation separately
Blood counts / renal / liver NCCP 00892 · SMPC	Baseline profiles	Every two weeks in both cycles	Before each cycle	As clinically indicated; more often if abnormal	Follow abnormalities before restart or after stopping

Care / check	Before first dose	Cycles 1–2	Cycles 3–6	Later treatment	Completion / change
ECG / heart rhythm NCCP 00892 · SMPC	Baseline ECG; check QTcF eligibility	Repeat around cycle 1 Day 14	Further ECG when clinically indicated	Repeat for symptoms, QT change or interacting medicines	No automatic restart after a QT-related hold
Electrolytes NCCP 00892 · SMPC	Potassium, calcium, magnesium and phosphorus	Before each cycle; correct abnormalities	Before each cycle through cycle 6	As clinically indicated	Review unresolved abnormalities
Bone / endocrine health NCCP 00892 · SMPC	Assess AI/ovarian function suppression (OFS) bone and menopausal risks	Support symptoms and adherence	AI/OFS and any injections follow their own plan	Repeat bone assessment according to risk	Longer endocrine and bone plan continues
Early-breast follow-up NCCP 00892 · SMPC	Confirm stage and prior local treatment	Assess new symptoms promptly	Follow early-breast surveillance plan	Mammography/follow-up as agreed; no automatic metastatic scan schedule	Continue surveillance

What each check is for

1 Before the first dose

Before first dose

Check eligibility, organ function and the exact treatment/support plan.

Ask / act: Confirm who arranges each check and who reviews it.

2 Early toxicity checks

Cycles 1–2

Important toxicity can begin before the first routine review.

Ask / act: Keep early appointments and use the agreed symptom-contact plan.

3 Benefit, tolerability and adherence

Later treatment

Longer-term treatment still needs clinical review.

Ask / act: Ask about symptom support, results and the next decision.

4 Completion or treatment change

Completion / change

The targeted medicine and endocrine partner may have different stop dates.

Ask / act: Confirm each medicine and the ongoing follow-up plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Baseline tests and ongoing tests have different purposes.

Before the first dose

full blood count (FBC), renal and liver profile; potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolytes and review QT/CYP3A interactions. Confirm stage, recurrence-risk criteria, prior treatment, AI and ovarian-suppression plan; reconcile the NCCP node-negative eligibility/exclusion conflict before selection.

During treatment

FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated. Repeat ECG around cycle 1 Day 14, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities; review fever, jaundice, palpitations/fainting, breathlessness and severe skin symptoms. Keep early-breast surveillance and bone/endocrine follow-up distinct from treatment-toxicity checks and from metastatic response imaging.

Practical treatment details

Supply and appointments

Request the next supply before tablets run out.

This oral treatment does not itself require a central line or infusion pump.

Support between appointments

Keep the oncology contact details and prescribed symptom medicines accessible.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Adjuvant ribociclib + endocrine therapy · NCCP 00892

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Ribociclib ON	 **Days 1–21** Once daily
Ribociclib OFF	 **Days 22–28** 7-day break
Endocrine therapy	 **Days 1–28** Continues on its own schedule
Cycle 1 ECG	 **Day 14** Around D14
Early blood tests	 **Day 14** Every 2 weeks in C1–2
Optional local review	 **Days 25–26** Local D25–26 window

Treatment bands: Show the planned medicine schedule, subject to review and dose changes.

Checks and restart: Keep the arranged blood / heart appointments. A calendar day alone does not authorise the next dose.

Symptoms: Can occur at any time; broad bands are not measured risk curves.

Day 25–26: An optional local pre-cycle review window, not a universal NCCP requirement. Early two-weekly tests and the Cycle 1 Day 14 ECG remain necessary.

Why these bands stay broad: Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00892 v2 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

Living with treatment

Supportive medicines and the endocrine partner have their own plan.

Nausea support

NCCP classifies ribociclib as oral moderate–high emetic risk. Agree preventive or rescue treatment under local policy after QT and CYP3A interaction review; no default QT-prolonging antiemetic is selected by this information card.

Heart rhythm and electrolytes

Attend the baseline and cycle 1 Day-14 ECG and scheduled electrolyte checks. Fainting, palpitations or severe dizziness need urgent assessment.

Blood and liver monitoring

Keep two-weekly blood tests during cycles 1–2 and pre-cycle tests during cycles 3–6. Report fever, jaundice, dark urine or new breathing/skin symptoms promptly.

AI and ovarian-suppression support

The aromatase inhibitor continues during the ribociclib rest week and generally after ribociclib completion. Discuss menopause, joints, sexual health, bone risk and any injection-site symptoms from ovarian suppression.

Completion and follow-up

Ribociclib has a planned maximum three-year course. Confirm the endocrine duration and early-breast surveillance plan separately; new symptoms should be assessed without waiting for a routine appointment.

When the plan changes

Continue, hold or stop

The team confirms the plan for each medicine and the next review. Do not restart a clinically held treatment from this educational timeline.

Practical support

Nausea support

Heart rhythm and electrolytes

Blood and liver monitoring

AI and ovarian-suppression support

Completion and follow-up

Taking tablets / attending treatment

Questions to ask

Which checks happen before and during treatment?

FBC, renal and liver profile; potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolytes and review QT/CYP3A interactions. Confirm stage, recurrence-risk criteria, prior treatment, AI and ovarian-suppression plan; reconcile the NCCP node-negative eligibility/exclusion conflict before selection. FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated. Repeat ECG around cycle 1 Day 14, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities; review fever, jaundice, palpitations/fainting, breathlessness and severe skin symptoms. Keep early-breast surveillance and bone/endocrine follow-up distinct from treatment-toxicity checks and from metastatic response imaging.

Do I need to enter dates?

No. This map shows general timing without personal information. Your hospital arranges actual appointments; ask who books each test and who reviews its result.

What if a dose is missed or treatment is held?

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription. A hold may affect one medicine rather than the whole combination; confirm each component.

What happens after treatment changes or ends?

At completion of the planned three-year ribociclib course, continue the separately agreed endocrine and follow-up plan. Keep the separately agreed disease follow-up, endocrine and bone-health plan.

Explain the plan back: Which medicine is taken when, which test is due next, who reviews it, and which symptoms mean you should call today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant ribociclib + endocrine therapy · supportive-care plan

Active medicines: Ribociclib + Aromatase inhibitor ± ovarian suppression

Emetogenic risk: Oral moderate–high

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP classifies ribociclib as oral moderate–high emetic risk. Agree preventive or rescue treatment under local policy after QT and CYP3A interaction review; no default QT-prolonging antiemetic is selected by this information card.
- Attend the baseline and cycle 1 Day-14 electrocardiogram (ECG) and scheduled electrolyte checks. Fainting, palpitations or severe dizziness need urgent assessment.
- Keep two-weekly blood tests during cycles 1–2 and pre-cycle tests during cycles 3–6. Report fever, jaundice, dark urine or new breathing/skin symptoms promptly.
- The aromatase inhibitor continues during the ribociclib rest week and generally after ribociclib completion. Discuss menopause, joints, sexual health, bone risk and any injection-site symptoms from ovarian suppression.
- Ribociclib has a planned maximum three-year course. Confirm the endocrine duration and early-breast surveillance plan separately; new symptoms should be assessed without waiting for a routine appointment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00892 · VERSION 2

Adjuvant ribociclib + endocrine therapy

The usual adjuvant starting dose is ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to three years. The aromatase inhibitor continues every day on its own plan; ovarian suppression is added when indicated.

· selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart rhythm / QT

Ribociclib

Baseline ECG and an ECG around Cycle 1 Day 14. Fainting or palpitations need urgent advice.

3 Low neutrophils / infection

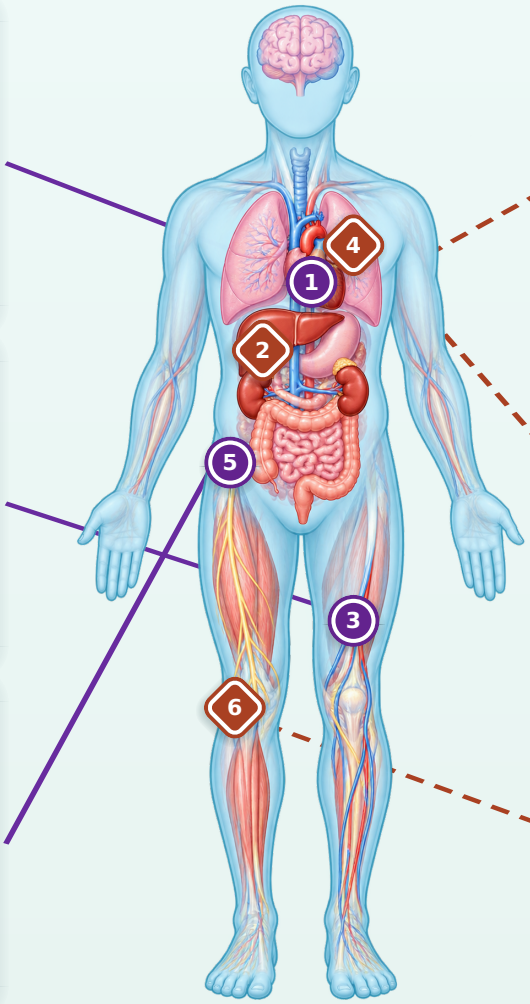
Ribociclib

Blood counts guide whether the next cycle can start. Fever, shivers or sudden illness: contact oncology immediately.

5 Severe rash or blistering

Ribociclib

A widespread painful rash, blisters or peeling skin needs urgent assessment.



2 Liver injury

Ribociclib

Keep liver blood tests. Yellow skin / eyes, dark urine or marked upper-abdominal discomfort: call promptly.

4 Lung inflammation

Ribociclib · uncommon but important

New cough or breathlessness needs prompt assessment, even between visits.

6 Joints and bone health

Aromatase inhibitor / ovarian suppression

Discuss joint symptoms and keep the agreed bone-risk and DXA plan.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

SACTCHECK · CHAPTER 04

Your record and contact card

Adjuvant ribociclib + endocrine therapy · National Cancer Control Programme (NCCP) 00892

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Nausea	☐	☐	☐	
Fatigue	☐	☐	☐	
Skin / nail changes	☐	☐	☐	
Muscle / joint pain	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly. Emergency symptoms: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Adjuvant ribociclib + endocrine therapy

NCCP 00892 v2

The usual adjuvant starting dose is ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to three years. The aromatase inhibitor continues every day on its own plan; ovarian suppression is added when indicated.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

https://sactcheck.com/patient/00892/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.21 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

SACTCHECK · CHAPTER 05

Sources and further information

Adjuvant ribociclib + endocrine therapy · National Cancer Control Programme (NCCP) 00892

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00892 v2 · official regimen ↗**
   https://healthservice.hse.ie/documents/6649/892_Ribociclib_Adjuvant.pdf
2. **EMA Kisqali current product information; checked 2026-10-25**
   https://www.ema.europa.eu/en/documents/product-information/kisqali-epar-product-information_en.pdf
3. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
4. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
5. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
7. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
8. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
9. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
10. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
11. **NATALEE · 4-year exploratory analysis (2025)**
    https://pubmed.ncbi.nlm.nih.gov/40996773/
12. **Novartis · Kisqali product information, updated September 2026**
    https://www.medicines.org.uk/emc/product/8110/smpc
13. **NATALEE primary report, Slamon et al 2024**
    https://pubmed.ncbi.nlm.nih.gov/38507751/
14. **NATALEE prespecified five-year report, Crown et al 2025**
    https://pubmed.ncbi.nlm.nih.gov/41320342/
15. **NATALEE six-year descriptive analysis, Slamon et al, Annals of Oncology journal pre-proof, 23 October 2026**
    https://www.sciencedirect.com/science/article/pii/S0923753426086400